

4.1 Medication Management

.....Revised 09/06/2018 Reviewed 09/06/2018

Policy Purpose

Medication is managed in accordance with applicable state and federal laws. The Health Services Director, Resident Care Director, Garden House Director and General Manager are responsible for monitoring Community compliance. The following guidelines apply unless superseded by specifically stated exceptions due to additional or enhanced licensure status.

Definitions

Medication

Any legend drug, controlled substance, over the counter, and consumer drug, preparation or substance used to treat a condition or relieve pain. All require an authorized prescriber's order.

PRN:

Medications prescribed on an as needed basis, (i.e., for pain relief)

Guidelines

Verification of Existing Medications upon Move-In

It is preferred that the resident obtain new prescriptions from the Community pharmacy whenever possible. The community cannot verify the quality of medication in unsealed containers.

Staff Assistance in Managing Medication

For team member assistance with medication:

- All medication is to be handled in accordance with company policy and the regulations set forth by state regulation.
- Resident's ability to self-manage medication will be evaluated (see Medication Services).
- Trained team members will provide medication assistance to those residents whose evaluation indicates a need for this service.
- Residents requiring medication administration must arrange for a qualified health professional (Example: LPN, LVN, RN) to provide this service if not permitted by state law or provided by the community.

GA: Uncertified Med Passers

Caregivers who are uncertified as medication aides or unlicensed caregivers may provide assistance with or supervision of self-administered medication only to those residents who are capable of self-administration or who require assistance with their medication and the resident(s) are able to exercise

judgment and choice about taking the medications and are able to articulate any physical reactions to the medications. All team members who assist in supervising medications for residents who are not capable of independent self-administration must be functionally literate (i.e. able to read medication labels and act in accordance with the instructions). Such team member assistance with or supervision of self-administered medication may only be provided for unit or multi-dose package medication prescribed for the particular resident and may include only the following tasks:

- *Taking the medication, in its previously dispensed properly labeled container, from where it is stored and bringing the medication to the resident;*
- *Reading the label, opening the container, removing a prescribed amount of the medication from the container, and closing the container, in the presence of the resident;*
- *Placing an oral dosage in the resident's hand or placing the dosage in another container where the resident requires assistance;*
- *Getting fluids such as juice, water or other for residents to consume;*
- *Applying topical medication;*
- *Returning the medication container to proper secured storage;*
- *Assisting the resident's use of an EPI pen where the resident has known severe allergies for which an EPI pen has been prescribed on condition that there is an established written protocol detailing how it is to be used and when. The protocol must include immediately calling EMS, 911 after any use of the EPI pen; and/or*
- *Documenting the observed dosage taken by the resident, the date, and the name of the team member supervising the medications, and PRN documentation.*

Basic training for team members assisting with self-administration will be documented and have the following curriculum components:

- (a) Community medication policy and procedure including action to take if concerns regarding resident's capacity to self-administer medication;*
- (b) How to read prescription label including common abbreviations;*
- (c) Providing right medications, to right resident, at right time, in right amount and right route, including how to measure various medications;*
- (d) Action to take when concern regarding meds are identified;*
- (e) Infection Control procedures relative to providing assistance with medications;*
- (f) Proper medication storage and disposal;*
- (g) Recognition of side effects and adverse reactions for the specific medications;*
- (h) Understanding of the common classifications of meds, typical side effects and adverse reactions and medication for which unlicensed staff may never provide assistance with or supervision of self-administrations; and,*
- (i) Proper documentation and record keeping using the MAR.*

Team members assisting with self-administration will have a competency evaluation upon hire in the form of a skills observation checklist prior to assisting with medication, which will be repeated at least annually.

GA: Certified Medication Aides (CMA)

For those residents, either who are not capable of self-administration of medication or choose not to self-administer medication with assistance or supervision, the community will provide medication administration services to the residents in accordance with physicians' orders, the needs of the residents and the rules stated in 111-8-63.20 Medications related to certified medication aides.

Certified Medication aides will only be allowed to do the following tasks:

- (a) Administer physician ordered oral, ophthalmic, topical, otic, nasal, vaginal and rectal medications;*
- (b) Administer insulin, epinephrine and B12 pursuant to physician direction and protocol;*
- (c) Administer medication via a metered dose inhaler;*
- (d) Conduct finger stick blood glucose testing following established protocol;*
- (e) Administer a commercially prepared disposable enema ordered by a physician; and/or,*
- (f) Assist resident in the supervision of self-administration of medication.*

All team members assisting or administering medication will have basic training (described herein) and those certified will be listed in good standing on the Georgia Certified Medication Aide Registry and have no record of being terminated for cause relating to the performance of medication aide tasks.

Certified Medication Aide Training must be done by an appropriately licensed health care professional (e.g. registered professional nurse, advance practice nurse, physician's assistant, pharmacist or physician) in accordance with the guidelines of the state mandated education components for medication training and state testing with signed affidavit of proctor who is registered with state; and skill competency check kept in CMA Notebook. Additionally, medication aides who have been certified for more than one year upon hiring must continue to have the knowledge and skill necessary to administer medication properly for that community and have a full competency skill check on community residents. The community will have standardized clinical skills competency checklists for each certified medication aide on file for renewal each year after the first year that are completed by a Georgia licensed RN, MD or pharmacist, who indicate in writing that the tasks observed are being performed competently.

Quarterly observations will be done by a licensed RN or pharmacist by conducting random medication administration observations to determine that the aides are administering medications correctly and in compliance with the stated regulations. Any issues will be reported to the community administration. Such training will be provided and documented by a licensed healthcare professional. This observation and review of med pass and MAR documentation will be done quarterly to verify all aides have at least one observation each six months.

GA: Proper Notice of Separation for Cause

When a medication aide is terminated for cause relating to the performance of medication aide tasks, the aide is provided the following by the community:

- (a) A separation notice that clearly describes the facts that support the termination;*
- (b) Written notice that being terminated for cause related to the administration of medication, if not successfully appealed in unemployment hearing will result in the loss of good standing on the Georgia Certified Medication Aide Registry; and,*
- (c) The loss of good standing on the CMA registry will make the aide ineligible for hiring as another certified medication aide by another ALC community.*

The community will submit to the Georgia Medication Aide Registry a copy of the separation notice for the certified medication aide only if the separation related specifically to the performance of medication aide tasks and the termination for cause has either been finally upheld by the DOL or the time for appeal for the notice has expired.

Storage of Medication by Community

Follow these guidelines regarding Community storage of and assistance with a resident's medications:

1. The General Manager may insist that the Community store and assist a resident with medication if:
 - The primary health care provider indicates the resident needs assistance with medication.
 - The nurse or prescriber determines the resident is incapable of appropriate self-management of medication (i.e. the resident forgets to take the medication, or is inclined to take too much medication). Resident rights may preclude such action, per state regulations. In such cases, careful documentation to limit the company's exposure to liability is mandatory.
2. Failure of a resident to cooperate with the Community's team members may deem the resident inappropriate for the Community. State authorities should be consulted in particularly difficult cases.
3. If the Community stores and assists a resident with prescription medications, the Community must also manage all other medications the resident takes, including all over-the-counter medication (laxatives, vitamins, antacids, aspirin, etc.) A resident may have control of one or more medications if the prescriber states, in writing, that it is permissible for the resident to have the medication in his/her possession. The written statement must specifically state which medication(s) the resident may have in his/her possession. Documentation will be placed in the chart in the Assisted Living Office (see Resident Records Management).
4. Medications controlled by the community should be stored in a locked area accessible only by authorized team members.

Requirements for Executing a Medication Program

The following supplies/requirements should be in place when setting up a medication program:

- A medication storage system, which should be obtained from the pharmacy at no cost.
- Paper and plastic soufflé cups.
- Medication storage devices may include cabinets, carts, cupboards or baskets (carts are to be used only if approved by the Vice President of Health Services).
- A prescription drug reference book and medical dictionary should be readily available as reference guides. The drug reference book should be no more than 2 years old.

GA: Reference material should be available to provide information on the medication, dosing, side effects, adverse reactions and contraindication for each medication being administered. Team members will be trained in its use.

- Documentation procedures for prescriptions, pharmacy records and prescription changes by prescriber. (See next section on Resident Information Documentation).

AZ: Semi-Annual review-For residents on the medication program, the medication orders will be reviewed and verified by a medical practitioner on a semi-annual basis.

GA: Quarterly Drug Regimen Review will be conducted by a pharmacist for each resident, and they will report any irregularities to the community administration with documentation. Additionally, in the review, they will remove for proper disposal any drugs that are expired, discontinued or in deteriorated condition, or where the resident for whom such drugs were ordered is no longer a resident. Lastly, they will establish and review policies and procedures for safe and effective drug therapy, distribution, use and control, as well as monitor compliance with established policies and procedures for medication handling and storage.

Resident Information Documentation Process

For the purposes of medication management, refer to the sections in *Resident Records Management* pertaining to the resident chart in the Assisted Living Office. Completed Medication and Treatment sheets are to be filed in the resident chart. An overflow file may be kept on the premises in a locked area. Upon discharge, the chart information is kept on file at the community for at least three years, or as required by state regulations.

Pharmacy Delivery and Storage

Prescription medications for residents receiving medication services must be kept locked in the medication room. Narcotics must be double locked. Pharmacy deliveries for residents managing their own medications, should be made to the resident's apartment. **Medications will not be kept at the front desk for pick-up by residents, families, or the pharmacy. They must be secured at all times.**

Only properly trained team members may handle resident medications, other than to move them from the front desk when delivered by the pharmacy to the medication room.

Prescription Sharing, Labeling, Transferring

Follow these guidelines related to the storage, transferring and labeling of medication:

Prescription medications may be used only for the resident for whom the medication was prescribed. Medications cannot be transferred from one resident to another, even if the prescription is the same.

- Prescription labels may not be altered in any way. Changes to dosage may be made only by an authorized prescriber and must be made in writing and delivered to a designated team member prior to changing the dosage. Prescriber's orders and Pharmacy Records of Prescriptions will be immediately noted in the chart and on the MAR. A label may be affixed to the prescription indicating dosage changes at the direction of the pharmacist. Team members may not alter the label.
- **A verbal prescriber's order will not be executed. All orders must be in writing.** All medications whether prescription or over the counter require a prescriber's order. A filled prescription is not a prescriber's order. PRN need to have how often to take, when and for what symptoms. A resident's medication may not be altered in any way (e.g. crushed or split) without a written prescriber's order.
- Medications must be stored in the manufacturers or pharmacy prepared packaging until prepared for delivery in accordance with state regulations
- Medications must be stored in the container supplied by the pharmacy, and with proper labeling. Containers include bubble packs, original vials, or other convenience delivery systems. We will not accept pre-filled "medi-sets" from family members, nor will we accept

medications in plastic bags, or other non-medical packaging. Emergency medi-sets may be acceptable as long as they are properly labeled with a pharmacy label.

Supply and Management of Medications

Team members trained in handling medications will be allowed to manage medications for residents. Those team members will follow the guidelines below:

- Residents are to receive service, treatments as well as medications and diet as allowable and prescribed. Timely prescription orders must be filled within 48 hours of receipt of notice of the prescription, or sooner if the prescriber indicates that a medication change must be made immediately. If the pharmacy does not have the medication needed for the immediate change available and has not obtained further directions from the prescriber, the community must notify the health care provider of the unavailability of the prescription and request direction. Refills of the prescribed medication must be obtained timely so that there is no interruption in the routine dosing. Where the community is provided with a new medication for the resident, the MAR must be modified to reflect the addition of the new medication within 48 hours or sooner if the prescriber indicates that the medication change must be made immediately.
- Maintain a minimum of five days' supply of medications.
SC: Maintain at least seven days' supply of medications.
- Every time a medication is delivered to a resident, it must be documented on the MAR whether the resident:
 - Took the medication,
 - Refused the medication, or
 - Accepted, but did not swallow, or otherwise use the medication.
- Medications may not be crushed or hidden in foods, unless ordered by an authorized prescriber or in accordance with state laws.
- A resident may not be forced to take a medication against his or her will. Refusal must be documented on the MAR and the health care provider notified of the refusal as soon as practical. It is important to consider why a resident may have refused a medication. Ask yourself:
 - Is the medication hard to swallow? Maybe there is a liquid form.
 - Does the medication taste bad? Prescriber approval is necessary to disguise the taste of the medication in juice or food.
 - Are there side effects? Be an advocate for the resident by calling the health care provider for evaluation of the necessity for the medication, or possible changes in dosage (titration).
 - How is medication being managed? Avoid demanding timing, standing too close, or appearing authoritative in your body language. Try passing the medication to the resident gently, and then retreating a short distance to "uninvade" the resident's space.

PRN and Over-the-Counter Medications

Over the counter (OTC) medications are viewed by state licensing regulations the same way as prescription medications and require a prescription. Vitamins, herbal preparations, and other consumer medications are also in this category. OTC medications must be stored in their original manufacturer

packaging if not packaged by the pharmacy. If not supplied with a pharmacy label, OTC medications must have the name and dose of the medication, manufacturer directions and the name of the resident visible on the original manufacturer container.

Medications to be used as needed (referred to as PRN medications) must meet the following guidelines:

- The resident must be able to indicate his or her own need for the medication.
- Detailed instructions must be included in the prescriber's order outlining the conditions under which the medication may be given and the allowable frequency.

Self-Management while Receiving Assisted Living Services

A resident receiving Assisted Living services who wishes to maintain his or her own medication, must meet the following guidelines:

- Provides resident medication history and agreement from the family. Safety is the key. The General Manager may overrule the wishes of the family, the resident or the prescriber if safety is an issue.
- Maintains safe and secure storage in the apartment, or the apartment door must remain locked when the resident is not present. Stores medications securely to prevent any roommates from having access.
- Understands policy and is periodically monitored regarding securing and not sharing medications.
- Receives periodic evaluation for any changes in ability or condition, or if problems occur in the medication routine (missed medications, wrong dosage, etc.)

When a Resident Is Away from the Community

If a resident receiving medication services leaves the Community for a short period of time, other than a hospital stay, the medications may be given to a responsible party with specific management instructions. The responsible party must sign the Release or Destruction of Medications form. This form includes a statement of understanding regarding the instructions and verifies the number of doses that have left the Community. Document in the resident chart and progress notes. If the stay is to be extended (i.e., a vacation), arrange with the pharmacy for a separate prescription to be used on the trip. Medications should be sent with the resident in pharmacy prepared packaging unless otherwise allowed by law. When the resident returns, medications should be counted and signed back in. Only medications in the pharmacy or sealed manufacturer prepared packaging will be accepted back.

WA: Family Assistance with Medications and Treatments

Family members may provide assistance with medications and or treatments, if the resident agrees and the family member providing the services submits a plan in writing to the Health Services Director, Resident Care Director, Garden House Director or General Manager. The plan must include:

- *Services that will be provided*
- *Name, dosage and time given for all current medications,*
- *A description of any treatment that will be provided,*
- *Name and phone number of the family member providing the services, to include an emergency contact number,*
- *An alternate plan should the family member providing the service become unavailable.*

The plan must be signed by the resident (if they are able), the General Manager or nurse Director, the resident's formal representative, and the family member providing the service. Should the plan of care change, the family member implementing the change must notify the General Manager or nurse Director immediately to review and update the current plan. Family members providing medication services cannot remove the resident's medications from the community while the resident is on the premises. Any change in medication to include new medications, change in dosage, time or frequency, and discontinued medications, the family member must notify the nurse Director.



Medication Management

Resident Care Manual

4.2 Medication Services.....Revised 09/06/2018 Reviewed 09/06/2018

Policy Purpose

Medications include prescription medications as well as over-the-counter medications. Medication service categories are determined for each resident in concert with the resident's primary health care provider, family and caregivers. The nurse will consider the resident's health, safety, ability and preference. Once the medication service category has been determined for the resident, it is documented in the chart. Each resident's medication service category will be re-evaluated upon any significant change of condition.

Definitions

Cognitive impairment

A disturbance in the mental activities associated with thinking, learning and memory.

MAR

Medication Administration (or Assistance) Record

Medication

Any legend drug, controlled substance, over the counter, and consumer drug, preparation or substance used to treat a condition or relieve pain. All require an authorized prescriber's order.

PRN

Medications prescribed on an as needed basis, (i.e., for pain relief)

Significant Change in Condition

Change in ability to perform one or more ADL's lasting longer than two weeks.

Guidelines

Medication Service Categories

The Community follows company policies and procedures for each medication service category, unless state law requires more stringent controls.

All residents are evaluated for ability to self-manage medications at move-in. Thereafter, only residents who manage their own medications will have their ability reviewed as part of routine re-evaluations. Residents who manage their own medication should also be reviewed for their ability to continue to self-manage medication if the resident:

- Has a significant change in condition,
- Becomes known to repeatedly forget to take medications,

- Does not store medications appropriately, or
- Is found to be taking medications more frequently than prescribed.

When the resident or responsible party requests that the community no longer manage medications, a re-evaluation of the resident's ability to self-manage will be completed.

Re-evaluation will include input from the primary health care provider for changes in services. Resident and/or the responsible party are to be notified of such changes. Requests for medication assistance are arranged upon entering the Community, or at any time thereafter by contacting the General Manager, Health Services Director, Resident Care Director, or Garden House Director.

If necessary, the resident will be assigned a new medication service category. All medications and blood glucose tests will be managed in accordance with appropriate state law. Exact definitions of the categories will differ slightly from state to state, but generally are as follows.

- Self-Management of Medication by Resident
- Self-Management of Medication with Assistance
- Administration of Medication (where permitted by law and performed by qualified staff)

It is the Community's responsibility to help make sure that each assisted living resident obtains his or her prescribed medications. All prescriber orders for any medication managed and controlled by the Community will be documented in the chart.

SC: *At each shift change, there shall be a documented review of the MAR's by outgoing staff members with incoming staff members that shall include verification by outgoing staff members that they have properly administered medications in accordance with orders by a physician or other authorized healthcare provider and have documented the administrations. Errors/omissions indicated on the MAR's shall be addressed and corrective action taken at that time.*

SELF-MANAGEMENT BY RESIDENT

The resident in this category is able to:

- Safely store medications,
- Fully understand the appropriate use of the medication, and
- Self-manage the medication according to prescribed dosage, time and any special instructions.

The resident may also be assigned this category if he or she cannot physically self-manage the medication but can correctly direct others to assist with opening the container and applying or instilling oral, skin, nose, eye and ear preparations as allowed by state regulations.

Team member documentation is not required unless Merrill Gardens team members are providing at least minimal assistance with medications.

GA: *Residents will have the right to self-medicate. The decision to self-medicate, and thereby not participate in the community's medication assistance program, will be documented on the Service Plan.*

SC: *Self-administering of medications by a resident is permitted only:*

- 1. Upon the specific written orders of the physician or other authorized healthcare provider, obtained on a semi-annual basis, or*
- 2. The Community shall ascertain by resident demonstration to the staff and document, at least quarterly, that s/he remains capable of self-administering medications.*

SELF-MANAGEMENT WITH ASSISTANCE

A resident in this category requires:

- Reminding or coaching,
- Guidance limited to opening the container, reading the label or prescriber's order, and/or explanation so that there is proper self-management, and
- Assistance with applying or instilling skin, nose, eye and ear preparations, within the limits set by state regulations.

Medications for residents in this category are stored to prevent access by residents. Documentation on a Medication Assistance Record (MAR) is required to include:

- Medication name and dosage,
- Time taken, or
- Resident's inability or refusal to take the medication according to the prescription. Observe whether the medication is taken, but do not force the resident to take the medication. Report the resident's inability or refusal to take the medication to the designated team member. Document on the back of the MAR. The appropriate team member will contact the resident's responsible party and/or health care provider for resolution. Team members assisting with medications must have appropriate training as required by state law.

ADMINISTRATION OF MEDICATION

A resident in this category:

- Cannot safely self-manage medication, or
- Cannot accurately self-perform a blood glucose test, and
- An authorized prescriber orders the medication to be managed by a licensed nurse or other authorized individual.

All services for this category are planned, directed and managed by a licensed nurse or authorized prescriber. The licensed nurse will train appropriate team members, keeping an attendance record, and placing documentation of training in team member records. Designated team members are to be observed and evaluated by the licensed nurse annually, or more often if needed, to assure proper management of the medications to meet the resident's needs.

Documentation is required to include:

- Medication name and dosage,
- Time taken, or

- Resident's inability or refusal to take the medication according to the prescription. Observe whether the medication is taken, but do not force the resident to take the medication. Document and report the inability or refusal to take the medication to the designated team member. The resident's responsible party and/or health care provider will be contacted for resolution.

SC: *Community staff members may administer medications, acting in a surrogate family role, provided these staff members have been trained to perform these tasks in the proper manner by individuals licensed to administer medications. Community staff members may administer injections of medications only in instances where medications are required for diabetes and conditions associated with anaphylactic reactions under established medical protocol. A staff licensed nurse may administer influenza and vitamin B12 injections and perform tuberculin skin tests. Sliding scale insulin is prohibited. Appropriately trained staff members may monitor blood sugar levels provided the community has a CLIA Waiver.*

Medication Packaging

Medications will be properly labeled and handled in accordance with current applicable laws and regulations. Residents are strongly encouraged to have their medications packaged in bubble packs, envelope systems, cassettes or other convenience packaging. The community may require a certain type of packaging for medications managed by community team members. The Health Services Director, Resident Care Director and/or Garden House Director should work with local pharmacists to determine the type of packaging that will be used in the community. Such systems help reduce the risk of human error and decrease the time it takes to set up medication. These medications should be kept in a locked medication cart or cabinet.

Pharmacists with repack licenses may repack the bottled medication into bubble packs or other convenience packaging. A pharmacy to pharmacy transfer of medication is usually required for repacks. They may also give the resident a credit for unused or unopened medication packages. The team members trained in medication policies and procedures should be familiar with local pharmacy rules.

Medication Storage

Medications must be stored in their original containers, which includes convenience packaging, with pharmacist-prepared or manufacturer's labels. Each resident's total medications are stored:

- Separately from other residents' medications
- Separately from food and toxic chemicals
- In an environment recommended on the medication label
- So medications are accessible only to designated team members or the appropriate resident
- Internal and external medications are kept separated from each other

STORAGE LOCATION AND ACCOMMODATION

All medication stored by the Community will be kept in a central safe and locked space that is not accessible to persons other than those authorized to handle medications. This area is to be kept clean, neat and organized at all times. Medication must be stored securely and inventoried appropriately to prevent loss and unauthorized use. Duplicate keys for all storage containers must be available on site for appropriate use. All medications except those required to be kept by a resident on his or her person

due to the need for frequent or emergency use must be stored by the community or by the resident if he/she desires, under lock and key at all times.

The majority of communities have a room allocated and designed for the specific purpose of storing medications. These rooms should contain locked cabinets and/or a medication cart, a small refrigerator and sufficient counter space to set up the medication, if applicable. Handwashing facilities should be in close proximity.

Nothing other than the medication, medication records and supplies should be stored in this area. The refrigerator should not contain anything other than medication that must be stored under refrigeration. The medication room should have adequate ventilation and the temperature should be kept at about 75° to 80°F or as required by state regulations.

If a resident receives his/her medication in bottles, the medication should be stored in individual bags, boxes or baskets that are clearly labeled with the resident's name. All of the resident's medication (except the PRN medication and any liquids, patches, drops or ointments) should be stored together. All medication should be kept in a locked cabinet.

PRN medication, liquid medication, drops, ointments etc. should be stored at the appropriate temperature, and accessible to all authorized team members.

SC: During nighttime hours in resident rooms, only medications which a physician or other authorized healthcare provider has ordered in writing for emergency/immediate use, e.g., nitroglycerin or inhalers, may be kept unlocked in or upon a cabinet or bedside table, and only when the resident to whom that medication belongs is present in the room.

Special consideration must be taken in the Garden House so that residents with cognitive impairment do not have access to medication. All medication must be stored in a locked area that impaired residents do not have access to.

Ordering Medication

Communities should always have an adequate supply of all centrally stored medication. It is the Community's responsibility to contact the appropriate persons to order the medication and to verify the accuracy of the medication when it is delivered to the Community. It is unacceptable to run out of medication.

PHARMACY SERVICES

Each Community will make arrangements with one or two pharmacies for ordering the majority of the residents' medications. The Community cannot insist that a resident use a specific pharmacy. However, the Community may outline the benefits and strongly recommend the use of pharmacies with which the Community has regular contact and a good working relationship. Residents utilizing the community's medication assistance program are required to have their medicines packaged in the manner chosen by the facility and required by the state.

Residents may use the pharmacy of their choice as long as the selected pharmacy will package medications in a method identical to the manner chosen by the facility and regulated by the state, which will be unit or multidose packaged and also having MARs that include diagnosis and major side effects

and allergies. The resident's chosen pharmacy must have an on-call system, 24 hours per day, 7 days a week. The only exception will be respite residents whose stay is less than 30 days and veterans who receive medications through the VA program. However, those receiving from the VA must agree to have repackaging meeting criteria above.

The Community works closely with the pharmacy to arrange for needed services to be provided. The General Manager, Health Services Director, Resident Care Director or Garden House Director may assist with any problems or concerns arising about the pharmacy. Some of the services a pharmacy may provide are:

- Packing services
- Centrally Stored Medication sheets
- Preprinted Daily Medication Records
- Same day delivery of medication
- Assistance with questions, problems and in-service training
- Cross checks of residents' medication to prevent harmful drug interactions

NV: *When providing medication assistance to residents, a physician, pharmacist or registered nurse who does not have a financial interest in the community will review the regimen of drugs taken by each resident of the community, including, without limitations, any over the counter medications and dietary supplements at least once every six months and provide a written report to the community to be reviewed and signed by the General Manager. A copy must be put in the resident's chart and their physician notified.*

SC: *an on-site review of the medication program by a pharmacist shall be conducted on at least a quarterly basis to help ensure the program has been properly implemented and maintained. A medication reference published within the previous 3 years is to be available to staff members assisting with or administering medications.*

ORDERING MONTHLY MEDICATIONS

Every pharmacy has a slightly different system for reordering medications. Become familiar with the reorder process of your particular pharmacies. Typically, medications may be automatically reordered on a regular basis. However, review a list of medications prior to the delivery date to verify that the pharmacy has current information on each resident.

ORDERING BOTTLED MEDICATION AND "NON-MONTHLY" MEDICATION

Every pharmacy has slightly different procedures for ordering medication. Become familiar with each pharmacy's specific procedures. There are, however, some standard procedures that must be followed when ordering bottled medication:

- Medications should be reordered from the pharmacy when down to a five day supply. If the resident's family is responsible for providing medication, contact them when the supply is down to 14 days.

- Medication orders must be recorded in the chart. Make sure to record the pharmacy and prescriber's name (if applicable).
- Do not wait until the last minute to contact the pharmacy or family regarding medications that have not been received. Call several days in advance of running out of the medications.
- Some medications have special ordering requirements. Example- pharmacy must have an original signed authorized prescriber's order to fill narcotics prescriptions.

Receiving and Tracking Medications

It is the Community's responsibility to verify that medications are correct before placing them in central storage.

NV: *A log of each medication received by the community for use by a resident must be maintained in the community.*

All medication must have a pharmacy label that includes the following information:

- Resident's name
- Name of authorized prescriber
- Medication care for those requiring special handling or storage
- Strength of the medication
- Quantity of the medication
- Date filled
- Expiration date
- Prescription number
- Instructions regarding the management of the medication
- Number of refills
- Hazardous drug labeling as indicated

OVER-THE-COUNTER MEDICATIONS

It may be possible to get labels for over-the-counter medications as well. If not, the prescriber's written orders must be on file in the chart for all over-the-counter and PRN medications. Do not accept any medication into central storage without written prescriber authorization. The resident's name must be written on all medications in manufacturers packaging that does not have a pharmacy label.

DAILY MEDICATION RECORDS

Medications should also be recorded on the Medication Record (MAR). If a medication is received bubble packed or in other convenience packaging, the pharmacist should be able to supply the Community with a pre-printed medication record. Otherwise, a team member will need to fill out a record for each resident on a monthly basis. The pharmacy may also provide blank records for residents using Veterans or HMO pharmacies.

The community utilizes a medication record for all residents included in the community's medication assistance program. The medication record requires staff notation and signature of medications distributed on its chart with date, time taken, refused or med error like missed dose identified. The General Manager is responsible for routine monitoring of the record to insure full and timely compliance with requirements of staff completion each time medication is indicated. Further the record will include:

- Name of resident;
- Known allergies;
- Name and phone of health care provider; and
- Name, strength and specific instruction for medication;
- A chart for staff who provide assistance or administration to record initials, time and date when medications are taken, refused or a medication error is identified (e.g. missed dosage). The staff providing the assistance or administration of medications are to update the MAR each time the medication is offered or taken.

GA: *MARS must also include key side effects or adverse reactions.*

Medication Changes

There may often be a change in a resident's medication. These changes should be made immediately and all records adjusted accordingly using the appropriate forms (see Resident Records Management).

A record of all communications with a resident's health care provider(s) should be noted in the chart. If a health care provider is requesting a change in the dosage, frequency or management of a resident's current medication, it must be communicated to the Community in writing with the prescriber's signature. Faxed or scanned copies are acceptable.

SC: *For changes in dosage, the new packaging shall be available in the community no later than the next administration time subsequent to the order. If a physician or other authorized healthcare provider changes the dosage of a medication, a label, which does not obscure the original label, shall be attached to the container which indicates the new dosage, date, and prescriber's name. In lieu of this procedure, it is acceptable to attach a label to the container that states, "Directions changed; refer to MAR and physician or other authorized healthcare provider orders for current administration instructions." The new directions shall be communicated to the pharmacist upon receipt of the order.*

NEW MEDICATION

If a resident receives a prescription for a new medication, either the prescriber or the Community should contact the pharmacy immediately to order the medication. When the medication is received, it should be checked according to procedures and placed into central storage. The information should be added to the chart and the MAR. (A line should be drawn through the days previous to the medication order.)

CHANGES IN MANAGEMENT INSTRUCTIONS

If the health care provider changes the management instructions for an existing medication, one of two procedures should be followed.

1. **Monthly Bubble Pack, Envelope or Convenience Packaging Systems:** Determine if the pharmacy will repack partially used cards. If so, return the entire card to the pharmacy with a copy of the prescriber's orders. The pharmacist will then verify the orders with the prescriber, repack the remaining days according to the new instructions and return the card to the Community. This should occur all in the same day, or within 24 hours.
2. **Bottles or Non-Monthly Bubble Packs, Envelopes or Convenience Packaging Systems:** An original written order from the health care provider should be requested within 48 hours. When received, the original order should be filed in the chart. A copy of the prescription should be secured to the bottle or card. Small labels may be obtained to affix to the bottle or card indicating an order change. The original instructions should not be altered or covered.

Documentation: In either case, document the change in the progress notes and the MAR. To make the change on the medication record, draw a line through the rest of the month on the existing record, go to the next available line and write in the new information. Draw a line to the date of the change and begin initialing from that point on.

DISCONTINUED MEDICATION

If the prescriber discontinues a medication, it must be noted in the progress notes. Written verification from the prescriber should also be obtained (fax or scan is acceptable). Then the bottle must be sealed and marked with the date the medication was discontinued. Fill out a *Release or Destruction of Medication* form. Draw a line on the Mar for the rest of the month. Write "DC", date and initial.

If the pharmacy has a repackaging license, the pharmacy may allow the return of any unused medication and will make the changes the prescriber has prescribed. Unused, discontinued medication that cannot be returned to the pharmacy must be destroyed per state requirements. Do not retain or give to resident or family unless required by state regulations.

GA: *Discontinued medications are considered resident's property and must be returned to the resident or responsible party. Medications left by the resident and unclaimed after notice to the above may be disposed in compliance with FDA guidelines and documented in the disposal record.*

SC: *Upon discharge of a resident, the community shall release unused medications to the resident, family member, or responsible party, as appropriate, and shall document the release with the signature of the person receiving the unused medications unless specifically prohibited by the attending physician or other authorized healthcare provider. Residents' medications shall be destroyed by the facility administrator or his/her designee when:*

1. *Medication has deteriorated or exceeded its expiration date;*
2. *Unused portions remain due to death or discharge of the resident, or discontinuance of the medication (may also be returned to the dispensing pharmacy). Medication that has been discontinued by order may be stored for a period not to exceed thirty (30) days provided they are stored separately from current medications. The destruction of medication shall be witnessed by the administrator or his/her designee, the mode of destruction indicated, and these steps documented. Destruction records shall be retained by the community for a period of two (2) years.*

Distributing Medication

All medication is to be distributed according to the prescriber's orders. It is the responsibility of the person distributing the medication to make sure the resident has taken the medication.

GA: Certified Medication Aides

Only a licensed nurse or a Certified Medication Aide may provide assistance or administration of medication to a resident in Georgia. Certified Medication Aides may only utilize unit or multidose packaging.

Certified Medication Aides may only:

- *Administer physician ordered oral, ophthalmic, topical, otic, nasal, vaginal and rectal medications.*
- *Administer insulin, epinephrine, and B12 pursuant to physician direction and protocol*
- *Administer medications via metered dose inhaler*
- *Conduct finger stick blood glucose testing following established protocol*
- *Administer a commercially prepared disposable enema ordered by a physician*
- *Assist residents in the supervision of self-administration of medications.*

GA: Uncertified or Unlicensed Caregivers

Uncertified and unlicensed caregivers may provide assistance with or supervision of self-administered medications only to those residents who are capable of self-administration, but choose to have community staff handle the medications for them.

Unlicensed or uncertified caregivers may only:

- *bring the labeled container to the resident, read, open, remove a prescribed amount of medication from the container, and close the container in the presence of the resident.*
- *return the medication container to secured storage.*
- *place an oral dosage in the resident's hand or in another container as requested by the resident.*
- *apply topical medications.*

BASIC GUIDELINES FOR MEDICATION SERVICES

Follow these general guidelines unless contradicted by state regulations:

1. The current months' Medication records (MARs) should be kept in a 3-ring binder. The binder should be organized with dividers for each resident. A photo of the resident should be kept with the MARs for identification assistance.
2. Medication should be distributed based on prescriber orders. Avoid distributing medication in the dining room.
3. The resident should be observed actually taking the medication. Medication should never be left on the table or nightstand for the resident to take later.
4. Medications may be refused by residents. In no event will a team member force in any manner a resident to accept medications. Team members will simply encourage the resident and remind them of the importance of medication usage as ordered by physicians. Medication

refusals will be documented in the community's medication administration record. Frequent medication refusals by residents will be communicated to the resident's personal physician and/or their responsible party.

5. Occasionally, the team member distributing medications may need to assist a resident in taking medication, if temporarily unable to self-manage. It may also be necessary if allowed by state regulations—with resident direction—to assist with drops or ointments if a tremor or poor eyesight prevents proper self-management. Assistance with managing injections or other invasive procedures are prohibited unless specifically permitted by licensure and only by qualified staff (RN's, LPN's or LVN's).

NV: State of Nevada regulations prohibit an employee of a residential facility to draw medication into a syringe or administer an injection.

6. Residents with cognitive impairment may need extra time to complete the tasks required to self-administer medication.

- Minimize distractions.
- Tell the resident it is time for medication.
- Ask the resident to perform one step at a time. Do not give another direction until the resident has completed the previous step.
- Some residents prefer to take one pill at a time.
- Offer a drink of water before and immediately after each pill.
- Learn the resident's preferences for taking medication.
- Medications should not be crushed or "hidden" in food or beverages unless the resident is aware they are taking medications and the physician has ordered it.
- Do not argue with the resident. If there is a problem, re-approach after a short break. Sometimes a different staff member may have success.
- If unable to get the resident to take medication after several attempts, notify the physician and document.

7. After distributing medication, the MAR should be initialed and signed by the person distributing the medication. Indicate the appropriate date and time, and/or what happened if resident did not take medication. Under "not available," indicate why the resident was not available.

"R" - Refused

"NA" - Not Available (This response requires additional information as to why the resident was not available)

Document additional information on the back of the MAR.

NV: The resident's physician must be notified within 12 hours of a medication being missed by a resident.

8. Team members providing medications are to document any unusual reactions to medications and provide such information to the resident or their legally authorized representative and any health care provider as appropriate.

Injectable Medication

All injectable medications must be self-administered or administered by an appropriately licensed person (i.e. and RN, LPN, physician's assistant, pharmacist, EMT, or medical doctor).

GA: *All injectable medications must be self-administered or administered by an appropriately licensed person (i.e. and RN, LPN, physician's assistant, pharmacist, EMT, or medical doctor) who is NOT an employee of the facility with two exceptions: administration of epinephrine or insulin. Both the administration of epinephrine and insulin may be administered by a certified medication aide and in the event of emergency EPI may be done with protocol by those assisting with self-administration.*

NV: *State of Nevada regulations prohibit an employee of a residential facility to draw medication into a syringe or administer an injection.*

PRN Medication

PRN medication refers to medication not given on a regular schedule, but rather given only when the resident requests it to relieve a specific symptom. The resident should be able to request PRN medication at any time (according to prescriber orders).

If the Community stores and distributes a resident's medication, it must also distribute PRN medication according to the prescriber's orders (unless written instructions from the prescriber state otherwise).

PRN medication is to be labeled like all other medications. The label must also clearly state the specific symptom for giving the medication, the exact length of time between dosages and the maximum number of doses that may be given in a 24-hour period.

If a PRN medication is given to a resident, it must be recorded on the MAR. The person giving the medication should initial the front of the MAR. The date, time given, medication given and reason given must be recorded on the appropriate place on the back of the MAR. The resident response to the PRN must also be documented.

CA: *If the resident's physician has stated in writing that the resident is able to determine and communicate their need for a prescription or nonprescription PRN medication, staff is permitted to assist the resident with self-administration of their PRN medication.*

If the resident's physician has stated in writing that the resident is unable to determine their own need for nonprescription PRN medication but can communicate their symptoms clearly, staff trained to deliver medications may assist the resident with self-administration under the following conditions:

There is written direction from a physician, on a prescription blank, specifying the name of the resident, name of the medication, specific symptoms which indicate the need for the medication, the exact dosage, the minimum number of hours between doses, the maximum number of doses allowed in each 24 hour period, instructions regarding a time or circumstance (if any) when it should be discontinued, and an indication when the physician should be contacted for a medication reevaluation.

If the resident is unable to determine their own need for a prescription or nonprescription PRN medication, and is unable to communicate their symptoms clearly, staff trained to deliver medication may assist the resident with self-administration under the following conditions:

Staff must contact the resident's physician prior to each dose describe the resident's symptoms and receive direction to assist the resident in self-administration of that dose of medication. The date and time of each contact with the physician and the physician's directions must be documented in the progress notes.

Hospice Comfort Kits

Where permitted by state regulations, communities may have comfort kits ordered by a hospice agency for a specific resident. Medications must be labeled according to pharmacy standards. The kit must be labeled with the resident's name. Comfort kits are to be centrally stored unless the resident has a family assistance plan (where allowed by state regulations). If stored in the resident apartment per a family assistance plan, the kit must be kept in locked storage accessible only by authorized persons identified in the plan. The apartment door must be kept locked when unoccupied.

Destruction of Medication

Medication is to be destroyed according to state guidelines. All completed medication records must be retained for three years after the resident moves from the Community. Destruction guidelines include:

- Expired medications
- Medications that have been discontinued for over 60 days
- Medications that were spilled
- Contaminated medications
- Medication not taken by the resident at the appropriate time
- Medication not taken by the resident upon moving from the Community

DESTRUCTION PROCEDURES

When a medication meets destruction guidelines, follow the guidelines below:

1. A designated team member should destroy all medication. The General Manager or Assisted Living Supervisor should witness the destruction. Both persons should sign or initial all paperwork involved, including the Release or Destruction of Medication form (or pharmacy provided form).
2. The medication should be counted, and the Release or Destruction of Medication form filled out completely. File the form in the chart.
3. Discontinued medication must be returned to the pharmacy or destroyed except where state regulations require other disposition.

GA: *Discontinued medications are considered resident's property and must be returned to the resident or responsible party. Medications left by the resident and unclaimed after notice to the above may be disposed in compliance with FDA guidelines and documented in the disposal record.*

4. Medications may be sent with the resident when moving out or temporarily leaving the Community. Use the Release or Destruction of Medication form. State the method of disposal being "left the Community" or "moved." Note the date, time, type and amount of medication taken. It should be noted just like a destruction and signed by the resident or responsible party.
5. In the case of death, medication must be destroyed. Never release left over or partial medication to the resident's family members or friends.

SC: Upon discharge of a resident, the community shall release unused medications to the resident, family member, or responsible party, as appropriate, and shall document the release with the signature of the person receiving the unused medications unless specifically prohibited by the attending physician or other authorized healthcare provider. Residents' medications shall be destroyed by the facility administrator or his/her designee when:

- 1. Medication has deteriorated or exceeded its expiration date;***
- 2. Unused portions remain due to death or discharge of the resident, or discontinuance of the medication (may also be returned to the dispensing pharmacy). Medication that has been discontinued by order may be stored for a period not to exceed thirty (30) days provided they are stored separately from current medications. The destruction of medication shall be witnessed by the administrator or his/her designee, the mode of destruction indicated, and these steps documented. Destruction records shall be retained by the community for a period of two (2) years.***

Physician Order Example

Sample Prescription

- 1 The patient's name and identifying information (age, address, date of birth)
- 2 Today's date
- 3 The name of the medication
- 4 The dosage of the medication
- 5 How many doses to be taken at once
- 6 By what route (oral, or otherwise) the medicine should be administered
- 7 How often the medicine should be taken and other details about how to take it, such as time of day, or with or without food
- 8 The number of doses to be dispensed at once (e.g., one month supply or more)
- 9 Number of refills
- 10 DEA number
- 11 Doctor's signature

Mary Smith, M.D.
Family Practice
Small Town, USA

Name John Doe Age 45
Address Small Town, USA Date 1/1/2018

Rx 3 4
Aspirin 325mg

5 6
Sign: 1 po qid prn

7
#12018

Refills 5 times
9

10
DEA #1234

11
Mary Smith, M.D.



Pharmacy Physician Order Sheet Example

HEARTLAND HOMECARE SVCS										Page 1 of 2
PE ID 2673		Telephone: (855) 331-0837		Facsimile: (785) 331-0876						
Patient: SAMPLE, JOE-SKITTLE				Location: HOLIDAY LIVING 123 GREAT CARE AVE HOMETOWN, KS 65555						
Due Date: 11.5.2011		Card Days: Annul		From: 11.5.2011 Thru: 12.4.2011		DOB: 01.01.1931 Room:				
Rx #	Drug Name	Rx Type	Schedule	Rx Start / Stop	Qty	Refills				
342222	ADVAIR DISKUS 100 50mc	DIS	CUSTOM	2/3/2011 2/7/2012	LOOSE					
SIG INHALE 1 PUFF BY MOUTH INTO LONG STRIDE A DAY FOR ASTHMA, FLUTICASON SALMETEROL										
342332	AZOPT 1% OP SUSP	DIS	CUSTOM	4/3/2011 4/13/2012	LOOSE					
SIG PLACE ONE DROP IN RIGHT EYE TWICE A DAY FOR GLAUCOMA, BRISOLANOL										
344885	BENZTROPINE 0.5MG TAB	DIS	CUSTOM	6/1/2011 6/8/2012						
SIG TAKE ONE 1 TABLET BY MOUTH THREE TIMES A DAY FOR EPS										
345223	CELEBREX 200MG CAP	DIS		6/1/2011 6/8/2012						
SIG TAKE ONE 1 CAPSULE BY MOUTH ONCE A DAY FOR PAIN, CELECOXIB										
100034	CLONAZEPAM 2MG TAB	DIS		6/1/2011 6/10/2012						
SIG TAKE 1 TABLET BY MOUTH ONCE A DAY FOR ANXIETY, CLONAZEPAM										
342335	DIVALPROEX ER 500MG	DIS		9/13/2011 9/13/2012						
SIG TAKE 1 TABLET BY MOUTH ONCE A DAY FOR MOOD STABILIZATION, EQ DEPAKOTE ER, DO NOT CRUSH										
342336	FLUTICASON 50mcg	DIS	CUSTOM	6/1/2011 6/7/2012	LOOSE					
SIG INHALE 1 PUFF IN BOTH NOSTRILS EVERY OTHER DAY FOR ALLERGIES, EQ FLOVASE										
342337	PRAMIPEXIDE 1.5MG TAB	DIS	TID	6/1/2011 6/6/2012						
SIG TAKE ONE 1 TABLET BY MOUTH THREE TIMES A DAY FOR SLEEP, ZUPRES (LEVET, RACETAM)										
342354	LAMICTAL 100MG TAB	DIS	AM/PM	6/1/2011 6/13/2012						
SIG TAKE ONE 1 TABLET BY MOUTH EVERY OTHER DAY FOR MOOD, LAMOTRIGINE										
Prescriber Signature _____ Date _____										



Apt #: Click or tap here to enter text. **Month:** Click or tap here to enter text.

[illegible]



Release or Destruction of Medications

This form is used to document disposition of medications removed from storage by the community. Check the appropriate box. Use more than one form if additional space is needed. Pharmacy provided forms may be used in place of this form for drug disposition.

☐ **Release of Medications-Short Term**
Medication for a resident going away from the Community for a short period of time (i.e., vacation)

☐ **Destruction of Medication**
Medication that is outdated or discontinued. (does not include Schedule II & III)

☐ **Release of Medications-Permanent**
Medication for a resident moving out

☐ **Disposition of Scheduled Meds-Permanent**
Schedule II & III medication released to resident or responsible party (Discontinued, move-out)

Resident's Name: _____ Apt #: _____

Drug Name	Quantity	Dose/Instructions	Reason for Med

Method of Destruction:

Medications must be destroyed by the Administrator or Designee (Resident Care Director) and another team member as permitted by state.

- ☐ Placed in Mail-Back receptacle ☐ Returned to Pharmacy ☐ Placed in Drug Destroyer type solution
- ☐ Mixed with coffee grounds and placed in trash ☐ Other:

Release of Medications:

I acknowledge receipt of the above medication(s), and understand the instructions for

- ☐ administering the medication(s) OR ☐ Properly disposing of the scheduled medications.

Medications released to: _____
Print Name above

Signature of Resident/Responsible party authorized for release of medication(s)

Date

Signature of person who released/destroyed the drug(s) Date

Administrator or Designee (Resident Care Director) Date



Medication Refrigerator Temperature Log

Procedure: Check and record refrigerator temperatures once day.

Refrigerators must be maintained at temperature of 36F to 46F degrees. If temperatures are not in the acceptable range, adjust refrigerator settings and recheck temperature within 4 hours.

Community:		Month	
Date	Time	Temperature	Name
1			
2			
3			
4			
5			
6			
7			
8			
9			
10			
11			
12			
13			
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4.3 Controlled Medications.....Revised 11/05/2018 Reviewed 11/05/2018

Policy Purpose

The Comprehensive Drug Abuse Prevention and Control Act of 1970 and the Controlled Substances Act are followed. Schedule II medications are stored in a locked non-removable container. Schedule II medications are counted at change of shift by the off-going and on-coming designated staff members. Schedule II medications must have a signed prescription by the licensed health care provider with prescriptive authority (prescriber) prior to the pharmacy filling the order for the medication. Records must be retained for at least two (2) years or as required by state law.

Note: Documentation for controlled medications per this policy is in addition to routine documentation of medication services on the medication assistance record (MAR).

Definitions

Controlled substance

A drug or other substance included in Schedule I, II, III, IV, or V of Section 812 of the Controlled Substances Act of 1996. For the purpose of this policy, controlled substances refer to scheduled medications.

Examples of Controlled Medications:

Schedule	Types of Medication	Storage	Documentation
Schedule I	No accepted medical use: Heroin, Marijuana, LSD, Ecstasy, Quaalude, Mescaline, Peyote, Psilocybin	Not in the community	Not in the community
Schedule II	High abuse potential: Fentanyl, Methadone, Oxycodone, Opium, Morphine, Demerol, Cocaine, Injectable Methamphetamine	Separate, locked, permanently affixed compartment within the locked medication storage area (double locked)	1. Controlled Substance Record Book 2. Shift count
Schedule III	Less abuse potential: Fiorinal, Amphetamine, Non-injectable Methamphetamine, Empirin with Codeine, Tylenol with Codeine, Anabolic steroids, CNS stimulants and depressants	Separate, locked permanently affixed compartment within the locked medication storage area (double locked)	1. Controlled Substance Record Book 2. Shift count

Schedule IV	Low abuse potential: Ativan, Restoril, Xanax, Meprobamate, Chloral Hydrate, Darvon, Dalmane, Librium, Valium, Phenobarbital	With resident's individual medications	No special documentation needed
Schedule V	Seldom abused: Paregoric, Antitussive preparations, Gabapentin	With resident's individual medications	No special documentation needed

Guidelines

Storing Controlled Medications

All Schedule II and III and designated controlled medications managed by the community are stored in separately keyed and locked storage areas within the community. This separately keyed locked storage area may be either a secure cabinet, drawer or medication cabinet, each having different locks and keys from the original locked container. This locked storage area is locked in another area (double locked). One team member per shift is responsible for the keys to the controlled medications storage area.

Ordering Controlled Medications

A signed written original prescription is required from the prescriber prior to filling a Schedule II prescription. A copy of a telephone order or a fax is not an original prescription. Schedule II drugs should be ordered in easy to count packaging, such as "bingo cards".

Counting Controlled Medications

Follow the process below to track controlled medications that are stored in the community:

1. A Controlled Substances Record Book is maintained for all Schedule II and III medications.
2. Each resident's prescription is entered on a separate page in the book immediately after delivery to the community. The entry should contain the following information:
 - a. Resident's full name
 - b. Prescription number
 - c. Pharmacy name
 - d. Name of prescribing physician
 - e. Name, strength, frequency and number of dosage units for the medication received
 - f. Method of administration
 - g. Date of receipt and signature of staff member receiving the medication
3. At the end of each shift, the off-going team member counts each controlled medication with the on-coming team member and compares the count with the amount recorded in the book.
4. Both team members sign the shift count in the book.
5. If a discrepancy is noted in the count, the team member is questioned for possible failure to sign the medication out when delivering to the resident. If the count is not reconciled, the Resident Care Director (RCD) is notified.

6. The RCD investigates, and if the count is not resolved, the General Manager is notified. The General Manager informs the Vice President of Operations.
7. The dispensing pharmacy is to be notified of unresolved count discrepancies. The pharmacy is responsible for any Drug Enforcement Agency or State Pharmacy Board reporting requirements.
8. An allegation of theft must also be reported to the appropriate state licensing agency and local law enforcement in accordance with state regulations.

Documenting Use of Controlled Medications

Documentation is an important part of the process of managing controlled medications. Follow t

1. Each time a Schedule II or III medication is removed from the locked area, it must be signed out on the correct page in the Controlled Substances Record Book.
2. Documentation is to include the following:
 - a. Date and time
 - b. Signature of team member who withdrew the medication
 - c. Amount of medication withdrawn
 - d. Amount of medication remaining after withdrawal
3. The controlled medication must also be signed out on the MAR upon consumption by the resident.
4. If an error in documentation is made, do not erase or obliterate it. Correct by drawing a single line through the incorrect entry, and initial it.
5. If a medication is contaminated by removal from the pharmacy packaging, refused or not given for any reason, it must be destroyed according to the section below, "Temporary Absence or Discharge with Medications."

Temporary Absence or Discharge with Medications

Residents may take currently prescribed controlled substances with them for temporary absences or upon discharge. When this occurs, document as follows:

1. Documentation should be completed as directed in the section above, "Documenting Use of Controlled Medications." The reason for giving the medication to the resident should be documented.
2. The resident must sign receipt of all medications taken with them. (see Release or Destruction of Medication form)
3. Receipt documentation must include:
 - a. Name of resident
 - b. Medication name, strength
 - c. Number of doses taken
 - d. Directions for use

4. Upon return from temporary absence, all controlled medications returned to the community in pharmacy packaging must be logged into the Controlled Substances Record Book as described in the section above, "Counting Controlled Medications."

Destroying Controlled Medications

1. Controlled medications cannot be returned to the pharmacy for credit.
2. Discontinued controlled medications shall have the respective controlled substances record closed with the following information:
 - a. "Discontinued", with date.
 - b. Remaining number
 - c. Disposition of remaining medication
3. Discontinued controlled medications are to be destroyed by the pharmacy or by placing in a pharmacy installed, managed and maintained collection receptacle for regulated scheduled medication disposal if the community is considered to be a long-term care facility. One supervisor level team member (generally the Resident Care Director) designated by the pharmacy may assist in changing the inner liner of the receptacle under the supervision of one pharmacy employee. Once removed, the sealed inner liner may be stored at the community for no more than three (3) business days in a locked cabinet or room.

CA: RCFEs are not considered to be long-term care facilities in California. Therefore, when a resident moves out or expires, controlled medications may only be disposed of by the person authorized to dispose of the resident's property. The pharmacy may not place a disposal receptacle in the community.

4. Controlled medications may not be destroyed by team members.
 - Discontinued controlled medications must be returned to the resident or the authorized representative to dispose of the resident's property. Only the resident or authorized representative may dispose controlled medications.
 - When a resident moves out, the controlled medications will be sent with them.
 - If a resident expires, the controlled medications must be released to the person authorized to dispose of the resident's belongings.
5. The community will provide information to the resident or expired resident's representative on how to dispose of the controlled medications per DEA guidelines, including take-back events, mail-back programs or collection receptacles as may be available.
6. The Resident Care Director and another team member (as allowed by state regulations) are responsible for documenting disposition of controlled medications stored by the community. Disposal of controlled medications should occur within three business days after discontinuation of use by the resident.

SC: The administrator must witness disposition of controlled medications within 30 days of discontinuance or move-out.

7. Disposal of Schedule II and III medications will be documented in the Controlled Substances Record Book as follows:

- a. Quantity
- b. Date discontinued
- c. Date disposed
- d. Method of disposition
- e. Signatures of both team members

AZ: Per R9-10-101(17) & (120) the following policy and procedure shall be in effect for residents other than those used as part of treatment for a terminal condition:

ADMINISTRATION OR ASSISTANCE WITH SELF-ADMINISTRATION

- ***Who: only trained Arizona Certified Caregivers and Licensed Nurses may administer or assist a resident in self-administration of opioid medications.***
- ***How and when: Prior to administration or assisting, the care team member will review the prescription label, MAR and the narcotic sign out sheet to verify correct order and count. The Team member will follow the physician's order on the MAR for indication of frequency, route and time.***
- ***The team member will ask the resident the location of pain and assign a rating based on a 1-10 scale with 10 being the highest. If the resident is unable to verbalize a pain number, the staff will use a face pain scale for input. The location and number will be documented on the MAR.***
- ***Monitoring a resident's response: The staff will follow up with the resident in the next 1-2 hours to determine the effectiveness using the 1-10 or face pain scale. The effect will be documented on the MAR.***

REPORTING OPIOID RELATED DEATH

If a resident's death may be related to an opioid, the General Manager is responsible for ensuring a written report is made to ADPH in a Department provided format within one day.



Date	Rx #	Rx #	Page No
Page Start Date	Transfer From Book	Page	By Nurse
Prescriber's Name		Drug / Med Name	
Prescriber's Name		Pharmacy Name	

Section 4 03a 05/01/2018

Section 4-3b 05/01/2018

4.4 Psychoactive Medications.....Revised 05/01/2018 Reviewed 05/01/2018

Policy Purpose

Residents have a right to be free of chemical restraints. Many undesired behaviors can be eliminated or reduced through environment and interventions other than the use of psychoactive medication. The use of psychoactive medications is discouraged; however, the resident's health care practitioner will determine whether their use is appropriate. Nothing in this policy should be construed as conflicting with the health care practitioner's decision to prescribe psychoactive medications.

Definitions

Chemical restraint

A psychoactive medication used for discipline or convenience and not required to treat the resident's medical symptoms.

PRN

Refers to medication ordered to be given under specific circumstances (as needed).

Pharmacological

The use of medication to have an effect on the resident.

Psychoactive medication

Medication that alters the mood, anxiety, behavior, cognitive processes or mental tension.

Target behavior

Specific resident behavior that the medication is being used to alter.

Guidelines

Prescriber Orders

Following are the requirements for prescriptions for psychoactive medications:

1. All psychoactive medications must be prescribed by a licensed health care practitioner with authority to prescribe the medication.
2. Medication orders must include:
 - a. Medication name
 - b. Dosage and frequency
 - c. Route of administration
 - d. Diagnosis to support use of the medication

e. Target behavior if PRN

Monitoring

The prescriber will determine the appropriateness and effectiveness of psychoactive medications.

A review of the medication must be included in the annual medical evaluation. A change in the resident's behavior should be treated as a change in condition and monitored as such.

Managing Undesired Behaviors

Many undesired behaviors can be reduced or eliminated without pharmacological intervention. (see *Behavior Management* policy)

4.5 Behavior Management.....Revised 05/01/2018 Reviewed 05/01/2018

Policy Purpose

In order to provide optimal care of residents with dementia or other cognitive impairment, key concepts of related behavior problems must be understood. Agitation, aggression, resisting assistance, wandering, incontinence, sleep disturbances, and emotional inconsistencies can become difficult management issues. In some residents, these disruptions can lead to overmedication and need for alternate placement. The focus should be on the medical, psychological, environmental and social factors that may contribute to undesired behaviors. Once pain, infection or other acute causes for the behavior have been ruled out, the philosophy of Merrill Gardens is to implement non-pharmacological measures for management of behavior problems that may arise among residents of the community.

Definitions

Non-pharmacological

Using other than medication to have an effect on the resident.

Psychological

Relating to the mind and its processes.

Guidelines

Wandering

Potential Causes	Management Strategies
Stress – noise, clutter, crowding.	Reduce excessive stimulation.
Lost – looking for someone or something familiar.	Provide familiar objects, signs, pictures; offer reassurance.
Restless, bored – no stimuli.	Provide meaningful activity.
Medication side effect(s).	Monitor, reduce or discontinue meds.
Lifelong pattern of being active or usual coping style.	Respond to underlying mood or motivation; provide safe area to move about.
Needing to use the toilet.	Institute toileting schedule; place signs or picture on bathroom door.
Environmental stimuli – exit signs, people leaving.	Remove or camouflage environmental stimuli; provide ID or alarm bracelets.

Difficulty with Personal Tasks

Potential Causes	Management Strategies
Task too difficult or overwhelming.	Divide task into small, successive steps.
Caregiver impatience, rushing.	Be patient, allow ample time or try later.
Cannot remember task.	Demonstrate action or task, allow resident to perform parts of the task that they can accomplish.
Pain involved with movement.	Treat underlying condition; consider pain medication or physiotherapy; assist the movement as needed.
Cannot understand or follow caregiver instructions.	Repeat request simply; state instructions one step at a time.
Fear of task – cannot understand need for task or instructions.	Reassure, comfort, distract from task with music or conversation; ask resident to help perform the task.
Difficulty initiating and completing a task.	Set up task sequence by arranging materials (such as clothing) in the order to be used; help begin the task.

Suspiciousness, Paranoia

Potential Causes	Management Strategies
Forgot where objects were placed.	Offer to help find; have more than one of same object available; have a list where objects should be placed; learn favorite hiding places.
Misinterpreting actions or words.	Do not argue or try to reason; do not take personally; distract.
Misinterpreting who people are; suspicious of their intentions.	Introduce self and role routinely; draw on old memory, connections; do not argue.
Change in environment or routine.	Reassure, familiarize, set routine.
Misinterpreting environment.	Assess vision, hearing; modify environment as needed; explain misinterpretation simply; distract.
Physical illness.	Evaluate medically.
Social isolation.	Encourage and provide familiar social opportunities.
Someone is actually taking something from the resident.	Verify the situation.

Agitation

Potential Causes	Management Strategies
Discomfort, pain.	Evaluate and manage sources of pain, constipation, infection or full bladder; check comfort of clothing.
Physical illness (such as UTI).	Evaluate medically; eliminate caffeine and alcohol.
Fatigue.	Schedule adequate rest; monitor activity.
Over stimulation – noise, overhead paging, people, radio, television, activities.	Reduce noise, stress; remove from situation; use TV sparingly; limit crowding.

Mirroring of caregiver's affect.	Control affect; model calm with low tone and slow rate; use support system and groups for outlet.
Overextending capabilities (resulting in failure); caregiver expectations too high.	Do not put in failure-oriented situations or tasks; understand losses; reduce expectations accordingly.
Resident is being "quizzed" (multiple questions that exceed abilities).	Avoid persistent testing of memory; pose one question at a time; eliminate questions that require abstract thought, insight, or reasoning.
Medication side effect.	Evaluate, monitor, and reduce medication if possible; monitor health concerns.
Resident thwarted from desired activity.	Redirect energy to similar activity; ask resident to help with meaningful activity; have diversionary tactics for outbursts; choose battles; evaluate whether behavior is merely irritating, rather than compromising resident safety or obstructing care.
Lowered stress threshold.	Simplify tasks, create calmness; lower expectations and demands; avoid arguments and reprimands.
Unfamiliar people or environment; change in schedule or routine.	Be consistent; avoid changes, surprises; make change gradually.
Restless.	Plan calming music, massage or meaningful activities; assign tasks that provide exercise.

Incontinence

Potential Causes	Management Strategies
Infection, prostate problem, chronic illness, medication side effect, stress or urge incontinence.	Evaluate medically.
Difficulty in finding bathroom.	Place signs, pictures on door, help ensure adequate lighting.
Lack of privacy.	Provide for privacy.
Difficulty undressing.	Simplify clothing; use elastic waistbands.
Difficulty seeing toilet.	Use contrasting colors on toilet and floor.
Impaired mobility.	Evaluate medically; treat associated pain; provide a commode; reduce diuretics when possible.
Dependence created by socialized reinforcement.	Provide increased attention for continence rather than incontinence whenever possible, even if time-consuming.
Cannot express need.	Schedule toileting; reduce diuretics and bedtime liquids when possible.
Task overwhelming.	Simplify; establish step-by-step routine.

Sleep Disturbance

Potential Causes	Management Strategies
Illness, pain, medication effect.	Evaluate medically.
Depression.	Antidepressant prescription, possible bedtime sedative.
Less need for sleep.	Schedule later bedtime; allow activities or tasks safely done at night; plan more daytime exercise.
Too hot, too cold.	Adjust temperatures.
Disorientation from darkness.	Use night-lights.
Caffeine or alcohol effect.	Reduce or eliminate alcohol; limit caffeine after noon.
Hunger.	Provide nighttime snack.
Urge to void.	Help ensure clear well-lit pathway to bathroom.
Normal age and disease related fragmentation of sleep.	Accept; plan for safety.
Daytime sleeping.	Eliminate or limit daytime naps; provide activity and exercise instead; for naps, use a recliner rather than a bed.
Fear of darkness, restless.	Provide soft music, massage, night light.

Inappropriate or Impulsive Sexual Behavior

Potential Causes	Management Strategies
Dementia related decreased judgment and social awareness.	Do not overreact or confront; respond calmly and firmly; distract and redirect.
Misinterpreting caregiver's interaction.	Do not give mixed sexual message; avoid non-verbal message; distract while performing personal care, bathing.
Uncomfortable – too warm, clothing too tight; need to void; genital irritation.	Check room temperature; assist with comfortable weather-appropriate clothing; help ensure that elimination needs are met; examine for skin problems.
Need for attention, affection, intimacy.	Increase or meet basic need for touch and warmth; model appropriate touch; offer soothing objects such as stuffed animals.
Self-stimulating – reacting to what feels good.	Offer privacy; remove from inappropriate place.

4.6 Medication Errors

.....Revised 05/01/2018 Reviewed 05/01/2018

Policy Purpose

Merrill Gardens team members strive to handle medications accurately according to prescriber orders. If a resident does not receive a medication as prescribed, this policy provides guidance to minimize errors and on appropriate follow up and documentation.

Guidelines

The community will follow established Merrill Gardens and pharmacy medication policies and procedures.

The community will contract with a licensed pharmacy to provide medication services according to standards of practice. The pharmacy will provide printed MARs, fill prescribed medication orders, and offer education and consulting services. Team Members who assist with medication services will be appropriately trained.

The Resident Care Director and Garden House Director will oversee the medication program in the community to help ensure medications are given to residents in accordance with prescriber orders.

Incidents and Errors

1. If an error is detected, an Incident Report will be completed. The prescriber and responsible party will be notified of the error.
2. The resident will be observed for any adverse effects and arrangements made for any necessary medical attention. Medication adverse reactions and medication errors are immediately reported to the Resident Care Director and Physician.
3. Corrective action will be taken to prevent recurrence which may include training, review of procedures or other appropriate action.

